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A. **Purpose** : To provide guideline instruction for admission of the patients to the inpatient care facility of the hospital.

B. **Scope**: This policy is applicable for indoor admission of all patients.

Abbreviations:

OP	–	Outpatients
OPD	–	Outpatient Department
IP	–	Inpatient patient
RC	–	Registration Counter
ED	–	Emergency Department.
EMO	–	Emergency Medical Officer
PV Ward	–	Private Ward
SPV Ward	–	Semi Private Ward
BHT	–	Bed Head Ticket.

C. **Policy**:

1. IP Admission from OPD:

Patients visit the OP clinics of the hospital for consultation.

Patient assessed by the doctor in the OP clinic and decision regarding the need to admit the patient in the IP facility of the hospital is taken by the treating consultant.

Patients are admitted only if the required medical care is available in the hospital. Incase of any discrepancy in the same, the Medical Superintendent is informed by the consultant doctor for clarification.

The doctor informs the patient and the relatives about the need for IP admission and indicates the same in the OP case sheet of the patient.

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The patient/relatives visit the IP registration counter of the hospital for admission of the patient.

The patient/relative provides the OP case sheet of the patient to the admission clerk who checks the same to ensure that IP admission is advised in writing by the treating consultant of the patient.

The admission clerk enquires from the patient about the their preferred ward which are :

i. Semi Private Ward.
ii. Private Ward.

2. Admission to Semi Private and Private Ward:

Filled in admission form along with the registration fee along with Rs. 15,000.00 advance is paid to the admission clerk.
Patient's details as indicated in the admission form are filled in the designated register and an inpatient number is allotted to the patient.

3. Admission from ED to the IP facilities:

Patient arrives directly in the Emergency Department of the hospital.

The patient is immediately assessed by the ED staff. The Emergency Medical Officer attend the patient and provides immediate resuscitation.

Incase the patient is to be seen by a specific consultant doctor for further treatment the consultant doctor of the specific specialty is informed. Patients case sheet is immediately prepared.

The consultant doctor attend the patient in the emergency ward, examines the patient and decides whether the patient is to be admitted in the IP facilities of the hospital.

If the patient is to be admitted in the IP facilities of the hospital, the consultant doctor indicates the same in writing in the patient case sheet.

The patient/relatives visit the IP registration counter of the hospital for admission of the patient..

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The on call consultant of the specific specialty is informed over phone by the EMO and is briefed about the patient's condition etc who instructs further treatment of the patient.

Incase the patient is serious and is to be seen immediately by the consultant doctor, the on call consultant will arrive in the hospital and examine the patient.

The on call consultant will decide as to whether to admit the patient in the hospital or to refer the same to the medical college for super specialty care.

If the patient is to be admitted, the same is indicated in the patients case sheet.

3. Procedure:

This involves the admission of the patient by the admission staff based on the information given by patient/ relative.

- The different categories of bed and the tariffs are explained to the relative.
- With the help of occupancy chart, if The room of choice/ward is available ,is allotted to the patient.
- Facilities are explained to the patient/relative.
- Registration is done
- All the patient's details are written in the IPD register (Pvt and WCL) with compulsory two mobile and one landline no.
- An IPD file is made.
- A general consent for treatment is taken, form is filled up by the relatives, in that at least 2 relatives mobile no. is taken. The declaration is to be signed by

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the patient or his relative / next of kin with the full name written clearly on the consent.

- Once the Performa is completed, it should be filed in the patient's record. The person on duty at admission counter must sign on the admission form for identification of originator, if the requirement arises.

- The patient's relative is then sent to the I.P billing department, with details of admission and the bed/ room allotted, for counseling and payment of deposit.

- A call is made to the ward regarding the new admission to make the room ready.

In case of an unidentified case

- Take the patient in casualty
- Provide the necessary emergency treatment
- Fill the MLC intimation, it has to be signed by CMO and countersigned by Medical Director or medical superintendent.
- Simultaneously do the registration of the patient as "unidentified case".
- After the MLC is done admit the patient.

Other Duties of front office staff -

- Contact various wards from time to time, (2 hourly) And keep yourself updated with the bed situation and expected discharges.

- The occupancy chart has to be updated and kept handy.

- Responding to enquiries regarding admission is duty of front office staff at this counter. Correct information expeditiously given, is of paramount importance.

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- In case of out station enquiries for admission, it is advisable to counter check with wards before confirming bed availability. This is even more significant when a patient is being transferred under emergency circumstances.

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